



Division of Provider Services
& Quality Assurance
P.O. Box 8059, Slot S404
Little Rock, AR 72203-8059

February 6, 2025

Derek Thompson, Administrator
Woodridge Of The Ozarks
2466 S 48th Street
Springdale, AR 72762

IMPORTANT NOTICE - PLEASE READ CAREFULLY

Dear Mr. Thompson:

On August 30, 2024, a Recertification survey was conducted at your facility by the Office of Long Term Care to determine compliance with Federal requirements for Psychiatric Residential Treatment Facilities participating in the Medicaid program(s). This survey found your facility was not in substantial compliance with participation requirements. Please refer to our letter, dated September 16, 2024.

A revisit was conducted on January 23, 2025, and your facility was still not in substantial compliance with the following participation requirement(s):

N141 , N142

Plan of Correction (PoC)

A Plan of Correction (PoC) for the cited deficiencies must be submitted within 10 calendar days of receipt of this letter to:

Breanna Marengo, Reviewer
OLTC, Survey & Certification Section
PO Box 8059, Slot S404
Little Rock, AR 72201-4608
(501) 320-6280
email to breanna.marengo@dhs.arkansas.gov.

A revisit will be authorized after an acceptable PoC is received. A completion date for each deficiency cited must be included. Your Plan of Correction must also include the following:

1. What corrective action(s) will be accomplished for those residents found to have been affected by the deficient practice;
2. How you will identify other residents having the potential to be affected by the same deficient practice and what corrective action(s) will be taken;
3. What measures will be put into place or what systemic changes you will make to ensure that the deficient practice does not recur; and,
4. Indicate how the facility plans to monitor its performance to make sure that solutions are sustained. The facility must develop a plan for ensuring that correction is achieved and sustained. This plan must be implemented, and the corrective action evaluated for its effectiveness. The plan of correction is integrated into the quality assurance system. At the revisit, the quality assurance plan is reviewed to determine the earliest date of compliance. If there is no evidence of quality assurance being implemented, the earliest correction date will be the date of the revisit; and
5. Include dates when corrective action will be completed. The corrective action completion dates must be acceptable to the State. Your facility is ultimately accountable for its own compliance. The plan of correction will serve as the facility's allegation of compliance. Unless otherwise stated on the PoC, the last completion date will be the date of alleged compliance.

Informal Dispute Resolution

In accordance with 42 CFR § 488.331, you have one opportunity to question deficiencies through an informal dispute resolution (IDR) process. **To obtain an IDR, you must send your written request to Health Facility Services, Arkansas Department of Health and Human Services within ten (10) calendar days from receipt of the Statement of Deficiencies.** The request must state the specific deficiencies the facility wishes to challenge. The request should also state whether the facility wants the IDR to be performed by a telephone conference call, record review, or a face-to-face meeting.

An incomplete informal dispute resolution procedure will not delay the effective date of any enforcement action or the requirement for timely submission of an acceptable plan of correction. Informal dispute resolution in no way is to be construed as a formal evidentiary hearing. It is an informal administrative process to discuss the findings.

Please submit your request to:

**IDR/IIDR Program Coordinator
Health Facilities Services
5800 West 10th Street, Suite 400
Little Rock, AR 72204
Phone: 501-661-2201
ADH.HFS@Arkansas.gov**

If you have any questions concerning this letter, please contact your reviewer.

Sincerely,

A rectangular box containing a handwritten signature in dark ink. The signature appears to be "Breanna Marengo" written in a cursive, somewhat stylized script.

Breanna Marengo, QA Team
DPSQA/Office of Long Term Care
Survey & Certification Section

bbm

cc: DRA

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR MEDICARE & MEDICAID SERVICES

PRINTED: 02/06/2025
FORM APPROVED
OMB NO. 0938-0391

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 04L120	(X2) MULTIPLE CONSTRUCTION A. BUILDING _____ B. WING _____		(X3) DATE SURVEY COMPLETED R-C 01/23/2025
NAME OF PROVIDER OR SUPPLIER WOODRIDGE OF THE OZARKS			STREET ADDRESS, CITY, STATE, ZIP CODE 2466 S 48TH STREET SPRINGDALE, AR 72762		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETION DATE	
{N 000}	Initial Comments Note: The CMS-2567 (Statement of Deficiencies) is an official, legal document. All information must remain unchanged except for entering the plan of correction, correction dates, and the signature space. Any discrepancy in the original deficiency citation(s) will be reported to the Dallas Regional Office (RO) for referral to the Office of the Inspector General (OIG) for possible fraud. If information is inadvertently changed by the provider/supplier, the State Survey Agency (SA) should be notified immediately. The facility was not in compliance with §483, Subpart G - Conditions of Participation for Psychiatric Residential Treatment Center.	{N 000}			
{N 141}	ORDERS FOR USE OF RESTRAINT OR SECLUSION CFR(s): 483.358(b) If the resident's treatment team physician is available, only he or she can order restraint or seclusion. This ELEMENT is not met as evidenced by: Based on interviews and record reviews, the facility failed to ensure a Physician's order for a chemical restraint was obtained for the actual amount of the chemical restraint that was given to 1 Client (Client #1) out of 3 case mix clients reviewed for chemical restraint orders. The findings are: Review of Client #1's Master Treatment Plan dated 10/01/2024, revealed the client had diagnoses including bipolar II disorder and post-traumatic stress disorder.	{N 141}			

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

Any deficiency statement ending with an asterisk (*) denotes a deficiency which the institution may be excused from correcting providing it is determined that other safeguards provide sufficient protection to the patients. (See instructions.) Except for nursing homes, the findings stated above are disclosable 90 days following the date of survey whether or not a plan of correction is provided. For nursing homes, the above findings and plans of correction are disclosable 14 days following the date these documents are made available to the facility. If deficiencies are cited, an approved plan of correction is requisite to continued program participation.

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{N 141}	Continued From page 1 A review of an ESI (Emergency Safety Intervention) packet for Client #1, dated 11/09/2024, signed by Registered Nurse (RN) #1 and the physician, revealed a Physician Order for - [Name brand antipsychotic medication] 5 mg (milligrams) and for [Name brand antihistamine] 100 mg, to be given IM (intramuscular) for aggression. The ESI signed by RN #1, indicated that [Name brand antipsychotic medication] 10mg and [Name brand antihistamine] 50mg was given to Client #1 by injection at 6:04 PM. A review of Client #1's MAR (medication administration record) dated 11/09/2024 at 6:04 PM, indicated [Name brand antipsychotic medication] 10mg and [Name brand antihistamine] 50mg was given as a (intramuscular) IM injection. On 01/22/2025, during an interview, the facility CEO (Chief Executive Officer) verified the medication administration record, and the Emergency Safety Intervention Packet indicated the medication dosages given were not the medication dosages ordered by the physician during the emergency safety intervention. The facility CEO reported that RN #1 was no longer employed by the facility with a termination date of 01/17/2025. On 01/23/2025 at 3:53 PM, during a telephone interview, the DON (Director of Nursing) reported she had been made aware of the medication error by the facility CEO and acknowledged the chart had been audited by nursing staff however, the error had not been discovered. Review of a facility policy regarding medication	{N 141}			

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{N 141}	Continued From page 2 administration with the last revision date of 01/2025, directed in part that nurses will administer all medications according to the physician's orders that specify the name of the drug, the dose, the frequency and the route of the administration. Review of a facility policy titled "Restraint or Seclusion Use" revised 1/2024, revealed in part a physician's order is to be obtained before a restraint or seclusion is implemented.	{N 141}			
{N 142}	ORDERS FOR USE OF RESTRAINT OR SECLUSION CFR(s): 483.358(c) A physician or other licensed practitioner permitted by the state and the facility to order restraint or seclusion must order the least restrictive emergency safety intervention that is most likely to be effective in resolving the emergency safety situation based on consultation with staff. This ELEMENT is not met as evidenced by: Based on interview and record review the facility failed to ensure that Physician's Orders for physical and chemical restraints were not received at the same time for 1 (Client #1) of 3 case mix clients reviewed for physician 's orders timing. The findings are: Review of Client #1's Master Treatment Plan dated 10/01/2024, revealed the client had diagnoses including bipolar II disorder and post-traumatic stress disorder.	{N 142}			

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{N 142}	Continued From page 3 A review of an ESI (Emergency Safety Intervention) packet for Client #1, dated 11/09/2024, signed by Registered Nurse (RN) #1 and the physician, revealed a Physician Order for a physical restraint was received at 5:59 PM. An additional order for [Name brand antipsychotic medication] 5 mg (milligrams), and for [Name brand antihistamine medication] 100 mg, to be given IM (intramuscular) for aggression. The order was received at 5:59 PM. The ESI signed by RN #, 1 indicated that [Name brand antipsychotic medication] 10mg and [Name brand antihistamine medication] 50mg was given to Client #1 by injection at 6:04 PM. The ESI revealed Client #1 was placed in a physical restraint at 5:57 PM and released at 6:18 PM. A review of Client #1's MAR (medication administration record) dated 11/09/2024 at 6:04 PM, indicated [Name brand antipsychotic medication] 10mg and [Name brand antihistamine medication] 50mg was given IM. On 01/23/2025 at 3:53 PM, during a telephone interview the DON (Director of Nursing) reported that the physician should be contacted for an order, for a physical restraint and then if indicated the physician should be contacted again for an order for a chemical restraint. The DON stated that the orders should not be obtained at the same time. A facility policy received on 08/26/2024 titled "Restraint or Seclusion use" with a revised date of "01/2024", was reviewed and read in part that a restraint is any manual method, physical or mechanical device, or drug used as a restriction to manage a client's behavior. The policy did not	{N 142}			

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{N 142}	Continued From page 4 address Physician's Orders received at the same time.	{N 142}			



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P.O. Box 8059, Slot S404
Little Rock, AR 72203-8059

March 13, 2025

Derek Thompson, Administrator
Woodridge Of The Ozarks
2466 S 48th Street
Springdale, AR 72762

Dear Mr. Thompson:

During the Complaint Investigation, Follow-Up/Revisit Survey conducted on March 10, 2025, your facility was found to be in compliance with program requirements. **Please email the signed CMS 2567 to breanna.marengo@dhs.arkansas.gov.**

If you have any questions, please contact your reviewer: **Breanna Marengo at 501-320-6280 or email to breanna.marengo@dhs.arkansas.gov.**

Sincerely,

A handwritten signature in black ink, appearing to read "B. Marengo", is placed over a grey rectangular background.

for Jeff Rosenbaum

Breanna Marengo, RN QA Team
DPSQA/Office of Long Term Care
Survey and Certification Section

bbm

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR MEDICARE & MEDICAID SERVICES

PRINTED: 03/13/2025
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OMB NO. 0938-0391

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NAME OF PROVIDER OR SUPPLIER WOODRIDGE OF THE OZARKS				STREET ADDRESS, CITY, STATE, ZIP CODE 2466 S 48TH STREET SPRINGDALE, AR 72762			
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LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE				TITLE		(X6) DATE	

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